



1 PATIENT DEMOGRAPHICS

Patient Name: Date of Birth:
Health Card #: Phone: Symptom onset / duration:

2 REFERRING PROVIDER

Name / Credentials: CPSO / NP Reg #:
Clinic / Office: Fax: Phone:

3 REASON FOR REFERRAL

☐ Pre-conception planning ☐ During pregnancy ☐ Postpartum / breastfeeding ☐ Medication safety review ☐ Second opinion
☐ Co-management

Clinical concern / additional detail:

4 REPRODUCTIVE STATUS & RHEUMATIC DIAGNOSIS

Reproductive status: ☐ Planning pregnancy ☐ Pregnant — GA EDD ☐ Postpartum ☐ Breastfeeding
Rheumatic diagnosis: ☐ SLE ☐ RA ☐ Antiphospholipid syndrome ☐ axSpA ☐ PsA ☐ Systemic sclerosis ☐ Myositis ☐ Vasculitis ☐
Undifferentiated CTD ☐ Other:
Disease activity: ☐ Remission ☐ Low ☐ Moderate to high Current treatment:

Well-controlled disease on pregnancy-compatible medication is the single biggest lever on outcomes; active disease is usually the larger risk.

5 RISK FLAGS & CLINICAL FEATURES (★ = high-yield for pregnancy risk)

Disease / obstetric history

☐ Active disease now or at conception ★
☐ Lupus nephritis, current or prior ★
☐ Prior pre-eclampsia / HELLP ★
☐ Prior pregnancy loss or stillbirth ★

Antibody-linked risk

☐ Prior thrombosis (APS) ★
☐ Prior neonatal lupus / heart block ★
☐ Anti-Ro positive and pregnant ★

High-risk organ involvement

☐ Pulmonary hypertension (high-risk) ★
☐ Significant renal disease ★
☐ Cardiac or pulmonary disease
☐ Interstitial lung disease

6 LABORATORY / ANTIBODIES (attach — pregnancy-critical panel)

KEY RESULTS — anti-Ro/SSA: ☐ Pos ☐ Neg anti-La/SSB: ☐ Pos ☐ Neg aPL (LA / aCL / anti-β2GPI): ☐ Pos ☐ Neg Platelets:
Creatinine:

Antibodies

☐ Anti-Ro / SSA* (fetal heart-block risk)
☐ Anti-La / SSB ★
☐ Antiphospholipid (LA, aCL, b2GPI) ★
☐ ANA
☐ Anti-dsDNA
☐ ENA panel

Disease / organ

☐ CBC
☐ Creatinine / eGFR
☐ Urinalysis + ACR (proteinuria) ★
☐ C3 / C4 complement ★
☐ Liver enzymes

Medications (list ALL)

☐ Flag: MTX, mycophenolate, leflunomide, cyclophosphamide, warfarin, ACEi/ARB
☐ Continue pregnancy-compatible agents

Current medications (list ALL, with dose):

Flag teratogens (MTX, mycophenolate, leflunomide, cyclophosphamide) and warfarin / ACEi / ARBs; these usually need stopping or switching before or during pregnancy.

Referring Provider Signature: Date:

Please attach: relevant lab results, imaging reports, and consultation notes. Outpatient / ambulatory referrals only.